

RESEARCH

Experiences of young university students undergoing rapid HIV testing

Experiencias de jóvenes universitarios ante realización de prueba rápida de VIH

Experiências de jovens universitários com o teste rápido de HIV

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Abstract

Introduction: HIV remains one of the biggest public health problems worldwide. To contribute to sustained intervention proposals and understanding of this phenomenon, it is necessary to address it from different perspectives. **Objective:** To gain an understanding of the experiences of young university students undergoing rapid HIV testing. **Methodology:** This qualitative, descriptive and phenomenological study was performed with 11 young university students who had experienced rapid HIV testing at least once. Convenience sampling was employed to ensure representativeness of the population until discourse saturation was reached. In-depth interviews were conducted using the following guiding questions: “What feelings did you experience before and during the rapid HIV test?”, and “How was your experience of having the sample taken?” An interpretative analysis was carried out in four phases. All stages of this study adhered to the ethical aspects of research. **Results:** Two categories emerged that expressed the meanings attributed to the experience of sample collection: “Normalizing testing to overcome barriers”, which refers to the obstacles to accepting testing that arise from knowledge and beliefs, and “Positive experience... what about the next one?” which refers to the positive experience of sample collection. However, certain beliefs can hinder early diagnosis. **Conclusions:** Based on young people's experiences, it is clear that HIV must be demystified, and the importance of routine testing must be promoted.

Key words: Rapid diagnostic tests; HIV; Life experiences; Phenomenology (DeCS).

Resumen

Introducción: El panorama del VIH sigue siendo uno de los mayores problemas de salud pública a nivel mundial, resulta necesario abordar este fenómeno desde diferentes perspectivas para contribuir a sustentar propuestas de intervención y comprensión de este fenómeno de estudio. **Objetivo:** Comprender las experiencias vividas ante la realización de la prueba rápida para detección de VIH en jóvenes universitarios. **Metodología:** Estudio cualitativo, descriptivo, fenomenológico, realizada en 11 jóvenes universitarios, con al menos una experiencia en la realización de la prueba rápida para VIH; muestreo por conveniencia considerando la representatividad de la población, hasta la saturación de los discursos. Se desarrollaron entrevistas en profundidad, con las preguntas orientadoras, ¿Cuáles fueron los sentimientos que te acompañaron antes y durante la realización de la prueba rápida para la detección del VIH? y ¿Cómo fue tu experiencia para toma de la muestra? Se realizó un análisis interpretativo en cuatro fases. Todas las etapas se apegaron a los aspectos éticos de la investigación. **Resultados:** Dos categorías expresaron los significados otorgados a la experiencia de la toma de la muestra: Normalizar la toma de la prueba para superar barreras, se refiere a cómo los conocimientos y creencias, surgen como obstáculo para la aceptación de la prueba; y la Experiencia positiva... ¿y la siguiente? se refiere a la experiencia positiva en la toma de la muestra, no obstante, ciertas creencias condicionan el diagnóstico precoz. **Conclusiones:** A partir de las experiencias de jóvenes, se subraya la importancia de la desmitificación de la enfermedad del VIH y las pruebas rutinarias accesibles.

Palabras clave: Pruebas de diagnóstico rápido; VIH; Experiencias de vida; Fenomenología. (DeCS).



Abstrato

Introdução: O panorama do HIV continua sendo um dos maiores problemas de saúde pública em nível mundial, sendo necessário abordar esse fenômeno a partir de diferentes perspectivas, com o objetivo de contribuir para sustentar as propostas de intervenção e compreensão desse fenômeno em estudo. **Objetivo:** Compreender as experiências vividas diante da realização do teste rápido para detecção do HIV em jovens universitários. **Metodologia:** Estudo qualitativo, descritivo e fenomenológico, realizado com 11 jovens universitários, com pelo menos uma experiência na realização do teste rápido para HIV; amostragem por conveniência, considerando a representatividade da população, até a saturação dos discursos. Foram realizadas entrevistas em profundidade, com as seguintes perguntas orientadoras: Quais foram os sentimentos que você teve antes e durante a realização do teste rápido para detecção do HIV? e Como foi sua experiência com a coleta da amostra? Foi realizada uma análise interpretativa em quatro fases. Todas as etapas seguiram os aspectos éticos da pesquisa. **Resultados:** duas categorias expressam os significados que este grupo atribui à experiência da coleta da amostra: Normalizar a realização do teste para superar barreiras, refere-se a como os conhecimentos e crenças surgem como obstáculo para a aceitação do teste; e Experiência positiva... e a próxima? Que expõe que, apesar da experiência, existem crenças que condicionam o diagnóstico precoce. **Conclusões:** destaca-se a importância de normalizar a coleta da amostra como parte de uma atividade regular e cotidiana para os jovens.

Palavras-chave: Teste de diagnóstico rápido; HIV; Experiências de vida; Fenomenologia (DeCS).

Introduction

Qualitative research is based on the idea that, as social beings, all people are both creators and consumers of a mental map of objective knowledge that influences their perception of the world ⁽¹⁾. In this respect, this study considers it relevant to approach the experiences of young people taking an HIV test subjectively, since symbolic aspects within social groups determine behavior, such as how health and illness are perceived.

Performing rapid HIV tests involves more than just a technical procedure. It also evokes complex emotional experiences that can affect decisions about testing, adherence to prevention protocols and perceptions of sexual health. Considering the above, phenomenology, for its part, is a qualitative research method that allows for the study of subjective aspects from the essence of phenomena. As they occur in their natural context, these phenomena are known as lived experience,



which is why there is interest in analyzing and understanding the meanings incorporated in the subject, based on their experience, before they are theorized ⁽²⁾.

Currently, HIV remains one of the biggest public health problems worldwide. By the end of 2022, it was estimated that 39 million people were living with the condition ⁽³⁾. According to UNAIDS, 1.3 million people became infected with HIV and 630,000 people died from HIV-related illnesses in that same year ⁽⁴⁾. In Mexico, HIV prevalence in the general population was estimated at 0.3% in 2023, meaning three out of every thousand people were living with the virus. Although this prevalence is considered low, it has remained unchanged for the past ten years, with 20,000 new infections reported in 2022 ⁽⁵⁾. Therefore, the importance of a timely diagnosis is emphasized. In this regard, rapid tests have proven highly effective in preventing and controlling HIV/AIDS by facilitating access to treatment and preventing new infections ⁽⁴⁾.

In the search for previous studies, results such as those of Piedrahita et al. ⁽⁶⁾ were identified. These results indicate that socio-cultural concepts surrounding HIV have become a barrier to accessing health services due to social stigma, religious significance and the perception of certain groups as being at “risk” of contracting the virus. A study in Latin America showed that young people from higher socioeconomic backgrounds were more likely to have accurate knowledge about HIV transmission and condom use. Regarding testing, inequalities were observed depending on the country ⁽⁷⁾. While Martín et al. ⁽⁸⁾ recognized that people's willingness to undergo testing was significantly influenced by their knowledge of the risk of infection, the importance they attach to testing for the detection of sexually transmitted diseases, and their confidence in the testing process. Although there are studies on HIV testing acceptance among young people, few have adopted a qualitative phenomenological perspective to explore the essence of the lived experience, particularly in Latin American or similar contexts, where socio-cultural factors such as *machismo* or religion may adjust these experiences.



Nursing, as a discipline focused on holistic care, plays a key role in administering rapid HIV tests and is responsible for pre- and post-test counseling, as well as prevention education. Reflecting on the experiences of young people will enable the development of more effective and empathetic strategies. This knowledge will enrich clinical practice and contribute to the integration of subjective dimensions into care models. The objective of this study was to understand the experiences of young university students undergoing rapid HIV testing.

Methodology

A qualitative, descriptive, and exploratory study was conducted to investigate the meaning of life experiences, considering that qualitative studies allow phenomena to be investigated in their natural environments, with the aim of explaining or interpreting events in terms of the meanings that people attribute to them. Additionally, this methodology is subjective, since it considers the researcher's observations ⁽⁹⁾. Moreover, it employs a phenomenological approach, explaining the nature of the phenomenon under study and focusing on observing the real world through reflections, stories, and accounts that participants deem significant ⁽¹⁰⁾.

Convenience sampling was employed through direct invitation until discourse saturation was reached. The participants were 11 undergraduate students from a public university who had taken at least one rapid HIV diagnosis test, whether provided by the university or not. Participants were selected based on their experiences related to the phenomenon under investigation, as well as their availability, willingness to discuss their experiences, and ability to reflect on and express their feelings. The process continued until sufficient information was attained in order to meet the research objective ⁽¹¹⁾.

In-depth interviews were conducted in a closed space during one or two face-to-face meetings between the researcher and the participant. The aim of the interviews was to gain an understanding of the participants' perspectives on their lives or events through a conversation between peers,



rather than an exchange of questions and answers ⁽¹¹⁾. As this was an unstructured, conversational process, the following two guiding questions were posed. However, others arose to allow for a deeper discussion and a more natural flow of conversation. “What were your feelings before and during the rapid HIV test?” and “How was your experience with the sample collection?” These interviews were recorded and subsequently transcribed into a word processor. Coding was carried out interview by interview and line by line.

This study was approved by the Ethics Committee of the relevant institution under the registration number FCS019/2024. The participant was provided with information about the study's objectives, how the information would be used, the risks involved, and the study's characteristics. In order to comply with the provisions of the General Health Law regulations on research ⁽¹²⁾, the informed consent form was read to the participant and their questions were answered.

In order to assess the information, the researchers followed the four steps suggested by Duque ⁽¹³⁾. First, they familiarized themselves with the interviews. To do so, they read the interviews independently and reflectively, taking notes in the margins about the information they considered noteworthy or relevant to the subject and purpose of the study. In the second step, once the initial readings were complete, the researchers worked together to identify common themes and they began to reduce the information. These initial steps enabled triangulation of experts, revealing similarities and contradictions to strengthen the quality of the research ⁽¹³⁾. In step three, emerging themes were categorized based on their logical connections or shared characteristics. Finally, step four defined the following categories (see Table 1).



Table 1. Categories and themes, 2025

Categories	Emerging Themes
Normalizing testing to overcome barriers	Incorrect and insufficient knowledge. Beliefs. Lack of risk perception. Feelings of fear and anxiety. Concern and doubts. Stigma.
Positive experience... what about the next one?	Uncertainty before the test. Positive experiences.

Source: Self-developed

Results

Of the participants, 55% were women and 45% were men between the ages of 17 and 22, attending a public university in southeastern Mexico. After the analysis, the following categories were identified: “Normalizing testing to overcome barriers” and “Positive experience... what about the next one?” The category “Normalizing testing to overcome barriers” emerged from the codes identified in the study group. This category focused on the obstacles to acceptance and testing posed by limited knowledge, lack of risk perception, expectations of results and beliefs related to the stigma of the disease.

Statements such as: *“I really didn't know there was a rapid test”* (E1); *“I didn't know where they were done”* (E7); *“I had been told that it was done one day before, that a blood sample was taken, and that the results would be given afterwards”* (E10). *“I had never had a rapid test done before, nor had I ever done one myself”* (E8); *“I thought they would take blood and that it would take hours”* (E10); and *“I thought they were going to take blood from my arm, not my finger. I didn't know it would be so quick. I thought it would take about an hour”* (E11) revealed a lack of information about the characteristics, timing and methods of sample collection.



Another important aspect was related to codes. Here, the interviewees were optimistic, assuming that their current state of health cancelled the need for screening tests or reduced their motivation to undergo them. For example, they said: “I don't really know much about the test. Precisely because I always consider myself healthy, I don't think it's necessary” (E9), and “*The confidence you have, saying: "It's not going to happen to me"*” (E10).

Participants mainly expressed fear, anxiety and uncertainty regarding their feelings before or during the test. The expectation of a positive HIV result was the main source of fear and anxiety: “*Fear of thinking what will happen if the results are not what I expect*” (E1); “*Fear that they might be positive and prevent it, and also the fear of not knowing whether I have it or not... I don't want to know. I feel fear and I don't know what I would do if the results were not what I expected*” (E2); “*Before the test, I experienced fear, anxiety and doubt*” (E3); “*To be honest, I was scared because I have a close relative with HIV*” (E5); “*It's scary to be tested and be told, 'You know what? You're positive'*” (E7); and “*I felt a little nervous. If I test positive, what am I going to do?*” (E11).

Other issues that arose from the interviews were personal health concerns, which led to diagnostic testing, and general concerns about health, particularly with regard to sexually transmitted diseases. It was this concern that prompted young people to seek information in the first place, as well as reassurance that they were healthy. Although the interviewees expressed the idea that they were doing things right, they still sought ways to confirm this. Some associated statements were: “*Sometimes condoms fail. I know I've been responsible, but something could have gone wrong*” (E1); “*With an active sex life, no matter how careful you are, there's always a fear that you might not be taking good care of yourself*” (E6); “*I know I protect myself in every possible way, but it never hurts to get an HIV test*” (E7); “*Even though I think I take all the necessary precautions, sometimes unexpected things can happen*” (E8); “*My friends are confident that they are not infected. They say... I don't have anything; I don't sleep around with just anyone. They believe that,*



because they are healthy now, they always will be” (E9); “It may not be 100% safe, even though I have been very cautious” (E10); “You never know what you're getting yourself into. Your hormones are raging, and you end up going to places you shouldn't. Lust is stronger than anything” (E2).

Finally, the stigma and fear of discrimination associated with the disease were addressed. These factors can discourage people, particularly young individuals still exploring their sexuality in terms of both pleasure and reproduction, from seeking an early diagnosis. *“People think that if someone is HIV-positive, they will infect others just by living with them. There is still a lot of discrimination in this regard, which creates tension among young people. They think, 'I'm not going to get tested; I'd rather not know if I have it or not,' because they are afraid of what others will say. It's as if someone said, “I'm going to get tested for HIV,” and the response is, “Why?” It's as if they saw it as a bad thing, when it's normal” (E4); “What if you bump into someone you know at the testing center? It's scary to say, “I've come to find out if I'm HIV positive.” Just saying that causes rejection in society; people see you as someone who is infected and should be avoided” (E5); “People don't come because they're afraid of being judged, of what others will say about them, that is, what if I really do have HIV?” (E8); “I thought: What if I test positive and then everyone finds out? That's scary, even though we know it's not contagious through contact. There are people who, if they know you're infected, won't come near you, and that's why we're afraid of being rejected” (E11).*

The category “Positive experience... what about the next one?” emerged from interviewees recognizing the importance of rapid HIV tests in combating the spread of the virus. These tests can provide quick and accessible results, facilitating early detection and appropriate intervention. They also expressed positive experiences after being tested. For this group of participants, the experience was a process that took them from initial uncertainty to acceptance of the decision that had been made. They recognized the necessity of the test, but emphasized their lack of prior knowledge



about what was involved. Some respondents highlighted the convenience of the location, its speed and ease of use, and the fact that it was free of charge. They also recognized that it is important to know that the environment is familiar and the confidence conveyed by the personnel. In addition, receiving negative results brought them relief and peace of mind.

However, a positive experience does not change the beliefs, stereotypes and values that allow young people to position within their social environment. This is why they ask, “What about the next one?”, as fear, apprehension and uncertainty will still be present in the next test, not because they are unfamiliar with it, but because of their expectations of the outcome.

The associated codes were: “I don't regret it. At first, I was a little hesitant to come, but since it was so quick, it didn't take up much of my time and I don't regret doing it” (E1); *“It's good that you came because these things are necessary, but you don't realize that”* (E2); *“It was pretty quick. I'm a bit scared of injections, but I didn't feel much pain”* (E5); *“They're free, so we should take advantage of the fact that they're here on a promotional campaign. The aim is to ensure that people do not feel judged if they ask for this test. I felt very good and calm; I was just nervous about the needle”* (E7); *“I had never had a rapid test before. It was quite a pleasant experience. It's good that the university is concerned about this. I am very happy with the results and the way the tests and protocol were handled”* (E8); *“I didn't know anything about this. At first, I thought I had to bring a special document or come on an empty stomach, but I didn't. They explained everything to me very clearly, which put my mind at ease”* (E9); *“When I saw the result later, I said, ‘Thank goodness, what a relief’”* (E10).

Discussion

The results obtained highlighted a lack of knowledge about the existence and technical aspects of rapid HIV testing. In addition, the experience prior to sample collection generated feelings that in some cases caused people to hesitate about taking the test. Another important consideration is the



stigma surrounding the disease, which influences the decision to get tested. It is important to reflect on the outcomes of the experience, including the disadvantages discovered and the benefits of rapid testing. While this is helpful for subsequent tests, it does not necessarily remove barriers to future testing.

Therefore, the results of some previous studies relating to emerging categories were discussed. These studies indicated that young people had an acceptable knowledge of the modes of transmission and risks of HIV and sexually transmitted diseases in general, however, they held misconceptions about diagnostic testing, effective treatment and test characteristics ^(14,16). These results were consistent with those of this study, in which participants also reported a lack of information about these tests, another study reported a low level of knowledge in more than half of the sample ⁽¹⁷⁾.

This study found that participants showed misperceptions, outdated information or information from informal sources, regardless of whether or not they had knowledge about the test, they used expressions such as *"I've been told"*. They also agreed on their lack of previous experience with testing, either due to the feelings it generated (such as fear) or because they had never felt the need to be tested *"I don't have anything, I don't mess with just anyone"*, which highlights the continuing stigma associated with the disease. All study participants attended voluntarily by invitation, other authors reported good acceptability of the rapid test, mainly if it was self-administered ⁽¹⁸⁻²⁰⁾.

It should be emphasized that this is not a question of acquired knowledge, but rather of group representations, such as beliefs about the disease, despite campaigns to prevent the stigma, these beliefs did not become normalized among the population of respondents. In this sense, a qualitative study revealed that participants' perceptions of HIV are shaped by the social environment which defines what is considered normal or tolerable ⁽²¹⁾. Likewise, the results obtained with a Latin American population with a similar age group, showed significant similarities, indicating that



nearly one-third of the population had been tested, mainly due to suspected infection, while the most frequent reasons for not being tested were lack of time and low perception of risk ⁽²²⁾.

In this study, there were statements such as “*my friends feel confident that they are not infected*” or “*I know that I take precautions in absolutely every aspect*,” responses that are related to the false belief that it will not happen to them and a low perception of the risk of infection, regardless of gender. In this sense, data found about the perception of risk of HIV infection, identified a category of invulnerability that exposes the idea that young people are not in danger ⁽²³⁾ however, a researcher found differences in this perception, where it was greater in men than in women, as well as the option “taking the test was a measure to protect oneself from HIV” was the second most frequently mentioned protective measure against HIV, despite the fact that more than half of the respondents in the aforementioned study had never been tested ⁽²⁴⁾.

Regarding the data emerging from aspects related to beliefs about the disease, young people said that they had not had the experience of being tested, but even so, however, they would wait for their peers to point them out. This highlights the stigma associated with the disease and the negative connotations surrounding it, which can lead to discrimination in the future. This stigma has been shown to influence decisions about testing, acting as a barrier to prevention, treatment, care and timely diagnosis ^(25–28). The interviewees' statements were in line with the idea of “*avoiding testing for fear of being judged or singled out by their peers*”.

When describing a positive and transformative experience with rapid testing, participants agreed that the key aspects were speed, free access, proximity, clarity of information and a non-judgmental environment, which fostered a pleasant experience and conveyed relief and satisfaction. The results were consistent with those of other studies examining users' experiences of rapid testing. These



studies have shown that confidentiality, flexible opening hours, pre- and post-test counseling, accessible locations and privacy are all factors that facilitate testing ^(29,30).

It is striking how the young people interviewed who underwent the rapid test can change their minds afterwards, but this does not necessarily imply a reconfiguration of their relationship with representation, since no new knowledge is presented. What does happen is that anxiety about the result dissipates, at least in the short term. Everything returns to normal, and the negative result can be viewed with satisfaction and peace of mind thanks to the test's various positive attributes, such as its speed, low cost and, above all, accessibility.

However, they do gain a new experience as they now understand the technical aspects of the test. Once they have overcome their anxiety and the associated stigma, they adopt a more objective stance towards the test. They may express codes such as *“happiness”*, *“they are necessary”*, *“I feel very calm”*, and *“nervousness only because of the needle prick”*, among others. This radically alters their perception of the test.

One limitation of this study was that participants were selected from a group of young people who had voluntarily attended the sample collection, which restricted the time available for gathering information. It was suggested that the information collected in the sociodemographic data sheet could be expanded to obtain quantitative data and provide a more complete picture of the population involved in the study.

Conclusions

This qualitative study, performed using a phenomenological approach, has enabled us to explore the experiences of young university students when taking the rapid HIV test, and to reveal the meanings that this group attributes to the experience. The two emerging categories: “Normalizing testing to overcome barriers” and “Positive experience... what about the next one?” reflect these experiences. The first category highlights obstacles to testing, such as fear, stigma, lack of



knowledge and misinformation. The second category emphasizes the positive aspects of the experience, such as the speed of the procedure, confidentiality, and support from healthcare professionals.

This study emphasizes the importance of considering young people's experiences in the context of sexual health and demonstrates that preventive and educational interventions should prioritize demystifying HIV and encouraging regular testing for the virus in locations accessible to young people. The results aim to provide qualitative evidence that can be used to design strategies tailored to the needs of this at-risk group.

It is particularly important to normalize HIV testing as part of the regular routine for young university students. Another consideration is to implement activities that increase demand for testing, such as providing specific information about testing and its benefits, and raising awareness of personal responsibility for healthcare.

This study emphasized the importance of nurses as key agents, as they are often responsible for promoting health and preventing diseases such as HIV.

Conflict of interest

The authors stated that there are no conflicts of interest.

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